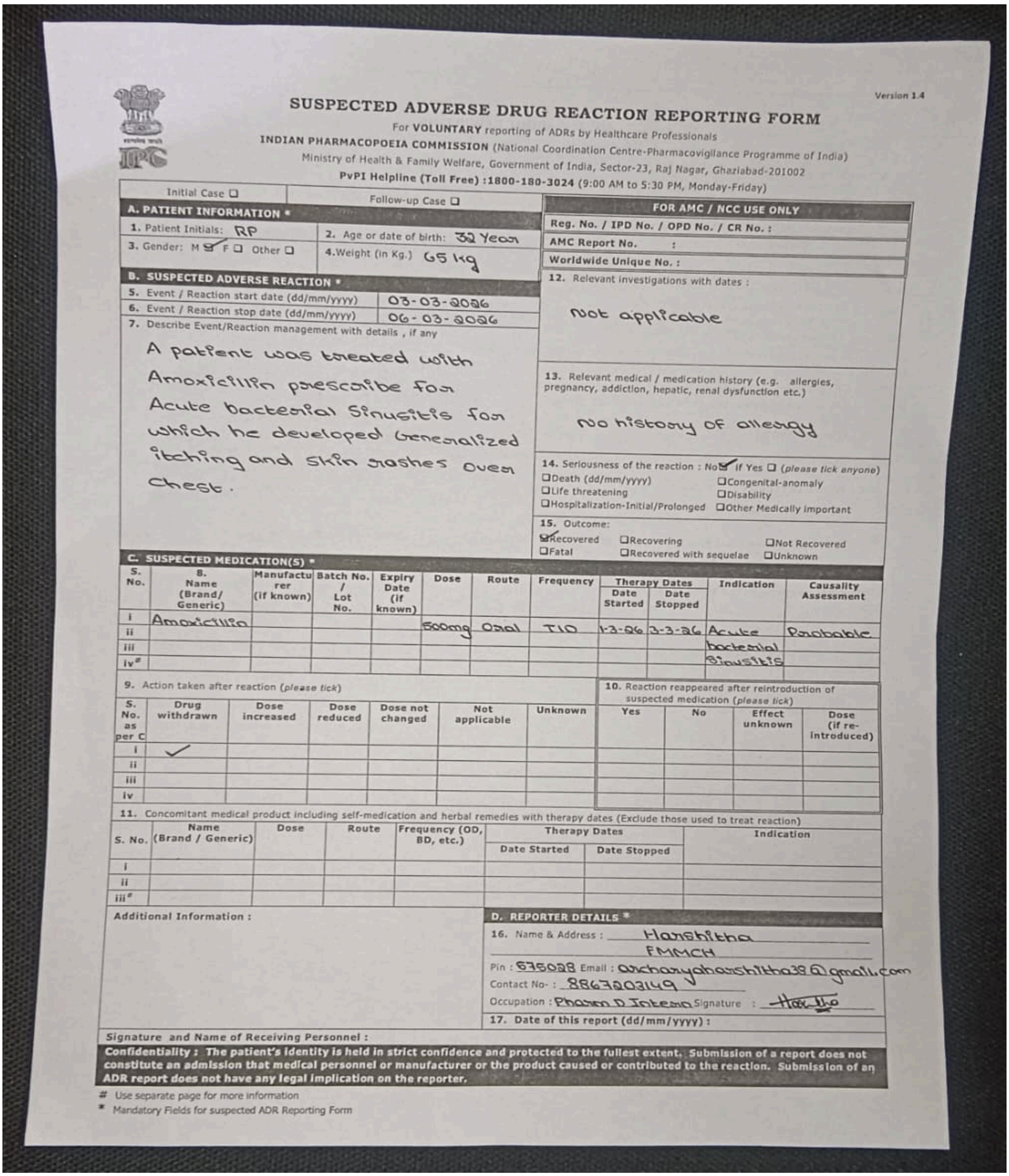
ADR REPORT FORM OF THE GIVEN CASE SCENARIO IN THE TUTORIAL VEDIO:



Version 1.4

SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) :1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☐ | Follow-up Case ☐

A. PATIENT INFORMATION *

1. Patient Initials: RP	2. Age or date of birth: 32 Year
3. Gender: M ☑ F ☐ Other ☐	4.Weight (in Kg.) 65 kg

B. SUSPECTED ADVERSE REACTION *

5. Event / Reaction start date (dd/mm/yyyy)	03-03-2026
6. Event / Reaction stop date (dd/mm/yyyy)	06-03-2026

7. Describe Event/Reaction management with details , if any

A patient was treated with Amoxicillin prescribe for Acute bacterial Sinusitis for which he developed Generalized itching and skin rashes over chest.

FOR AMC / NCC USE ONLY

Reg. No. / IPD No. / OPD No. / CR No. :

AMC Report No. :

Worldwide Unique No. :

12. Relevant investigations with dates :

not applicable

13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)

no history of allergy

14. Seriousness of the reaction : No ☑ If Yes ☐ (*please tick anyone*)

☐Death (dd/mm/yyyy) ☐Congenital-anomaly
☐Life threatening ☐Disability
☐Hospitalization-Initial/Prolonged ☐Other Medically important

15. Outcome:

☑Recovered ☐Recovering ☐Not Recovered
☐Fatal ☐Recovered with sequelae ☐Unknown

C. SUSPECTED MEDICATION(S) *

S. No.	8. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication	Causality Assessment
i	Amoxicillin				500mg	Oral	TID	1-3-26	3-3-26	Acute bacterial Sinusitis	Probable
ii											
iii											
iv#											

9. Action taken after reaction (*please tick*)

S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown
i	✓					
ii						
iii						
iv						

10. Reaction reappeared after reintroduction of suspected medication (*please tick*)

Yes	No	Effect unknown	Dose (if re-introduced)

11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)

S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication
i							
ii							
iii#							

Additional Information :

D. REPORTER DETAILS *

16. Name & Address : Harshitha
FMMCH

Pin : 675028 Email : acharyaharshitha38@gmail.com

Contact No- : 8867203149

Occupation : Pharm D Intern Signature : Harshitha

17. Date of this report (dd/mm/yyyy) :

Signature and Name of Receiving Personnel :

Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.

Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form

ADR REPORT FORM OF THE CASE SCENARIO WHICH I HAVE SELECTED:

Version 1.4

SUSPECTED ADVERSE DRUG REACTION REPORTING FORM

For **VOLUNTARY** reporting of ADRs by Healthcare Professionals

INDIAN PHARMACOPOEIA COMMISSION (National Coordination Centre-Pharmacovigilance Programme of India)

Ministry of Health & Family Welfare, Government of India, Sector-23, Raj Nagar, Ghaziabad-201002

PvPI Helpline (Toll Free) :1800-180-3024 (9:00 AM to 5:30 PM, Monday-Friday)

Initial Case ☑ | Follow-up Case ☐

FOR AMC / NCC USE ONLY

Reg. No. / IPD No. / OPD No. / CR No. : 20086009

AMC Report No. :

Worldwide Unique No. :

A. PATIENT INFORMATION *

1. Patient Initials: PHCH
2. Age or date of birth: 90 Year
3. Gender: M ☑ F ☐ Other ☐
4. Weight (in Kg.)

B. SUSPECTED ADVERSE REACTION *

5. Event / Reaction start date (dd/mm/yyyy): 24-09-2026
6. Event / Reaction stop date (dd/mm/yyyy): 27-09-2026
7. Describe Event/Reaction management with details , if any

A Patient was treated with Inj Mannitol for Acute CVA for which the patient developed Decreased Serum Potassium level. (Hypokalemia).

12. Relevant investigations with dates : Sr K+ level :
23-09-2026 - 3.6 mmol/L
24-09-2026 - 3.2 mmol/L
25-09-2026 - 3.2 mmol/L
26-09-2026 - 3.0 mmol/L
27-09-2026 - 3.6 mmol/L

13. Relevant medical / medication history (e.g. allergies, pregnancy, addiction, hepatic, renal dysfunction etc.)
K/C/O : Alzheimer's disease, Old IHD, BPH.
Diag : Acute CVA - GC bleed extending to right corona radiata with left hemiparesis, Sepsis - Aspiration Pneumonia.

14. Seriousness of the reaction : No ☑ if Yes ☐ (*please tick anyone*)
☐Death (dd/mm/yyyy) ☐Congenital-anomaly
☐Life threatening ☐Disability
☐Hospitalization-Initial/Prolonged ☐Other Medically important

15. Outcome:
☑Recovered ☐Recovering ☐Not Recovered
☐Fatal ☐Recovered with sequelae ☐Unknown

C. SUSPECTED MEDICATION(S) *

S. No.	8. Name (Brand/ Generic)	Manufacturer (if known)	Batch No. / Lot No.	Expiry Date (if known)	Dose	Route	Frequency	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication	Causality Assessment
i	Inj Mannitol (Mannitol)				100ml	IV	BD	24-09-	25-09	Acute CVA	Probable
ii											
iii											
iv#											

9. Action taken after reaction (*please tick*)

S. No. as per C	Drug withdrawn	Dose increased	Dose reduced	Dose not changed	Not applicable	Unknown
i	✓					
ii						
iii						
iv						

10. Reaction reappeared after reintroduction of suspected medication (*please tick*)

Yes	No	Effect unknown	Dose (if re-introduced)

11. Concomitant medical product including self-medication and herbal remedies with therapy dates (Exclude those used to treat reaction)

S. No.	Name (Brand / Generic)	Dose	Route	Frequency (OD, BD, etc.)	Therapy Dates: Date Started	Therapy Dates: Date Stopped	Indication
i	Neb Duolin (Levosalbutamol 500mcg + Ipratropium)	1.25mg +	Mask	Q8hr	19-09-2026	10-10-2026	Sepsis - Aspiration Pneumonia.
ii							
iii#							

Additional Information :

On 24-09-2026 - Syp Potklor 10ml TID given.

On 25-09-2026 - Inj KCl @ Amp in 500ml fluids @ 100cc/hr.

D. REPORTER DETAILS *

16. Name & Address : Harshitha, FMMCH

Pin : 535028 Email : acharyaharshitha38@gmail.com

Contact No- : 8867203149

Occupation : Pharm D Intern Signature : Har-tha

17. Date of this report (dd/mm/yyyy) : 29-09-2026

Signature and Name of Receiving Personnel :

Confidentiality : The patient's identity is held in strict confidence and protected to the fullest extent. Submission of a report does not constitute an admission that medical personnel or manufacturer or the product caused or contributed to the reaction. Submission of an ADR report does not have any legal implication on the reporter.

\# Use separate page for more information

* Mandatory Fields for suspected ADR Reporting Form

CASE SCENARIO THAT I HAVE SELECTED TO WRITE THE ADR FORM:

CASE SCENARIO:

Patient Name: P H C G

Age: 92 Year

Gender: Male

Allergy Status: No Known Allergy

Past Medical History: Alzheimer’ s Disease, Old IHD, BPH

Present Illness: Acute CVA – GC bleed extending to right corona radiata with left hemiparesis, Sepsis – Aspiration Pneumonia (Resolving).

Past Medication History:

T. ESCOSPRIN AV (75/10MG) 0-0-1
T. VELTAM S 0-0-1
T. DAPAGLIFLOZIN 5MG 0-1-0
T. CIDMUS 50MG 1/2-0-0
T. CARDIVAS CR 10MG 1-0-0
T. OXRA 5 MG 1-0-0

On 24/09/2026 The Patient Serum Potassium Level Declined to **3.2 mmol/L**

On 25/09/2026 The Patient Serum Potassium Level Declined to **3.2 mmol/L**

On 26/09/2026 The Patient Serum Potassium Level Declined to **3.0 mmol/L**

LABORATORY DATA:

Physical Examination:

Blood Pressure: 110/70 mmHg

Heart Rate: 120 BPM

Respiratory Rate: 22CPM

MANAGEMENT:

On 24/09/2026 She was started with Syp Potklor 10ml TID

On 25/09/2026 She was treated with Inj KCl 2 AMP in 500ml fluids @ 100CC/hrs

The Decreased Serum Potassium Level (Hypokalemia) was resolved on 27/09/2026